



Peace of mind for families of
hospitalized patients.

The family

“Maria, 78, was admitted to hospital in Lisbon on Monday.”



Pedro lives in Berlin.

Visiting isn't an option.



Ana works in Lisbon.

Doctors take calls 12:00–13:00, in the middle of her workday.



**When they call,
nobody picks up.**

The doctors are
with patients.

“So they call again. And again.”

This happens every day, at up to 3300
hospital beds in Portugal.¹

¹ ~1.2 million hospital admissions in Portugal in 2024 (INE, Estatísticas da Saúde).

The hospital

22 families

3 doctors

1 phone hour

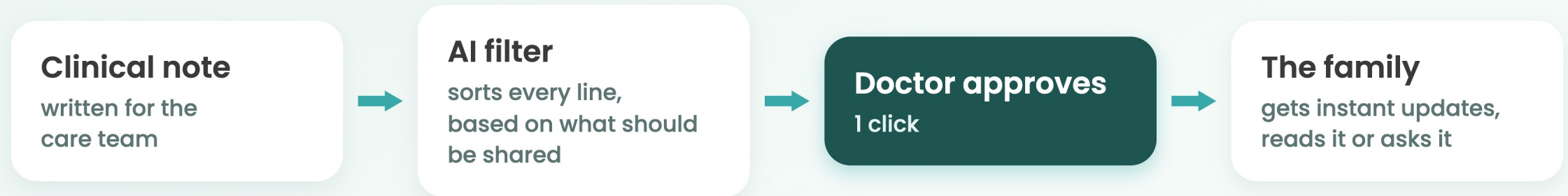
Nurses may only say **"She's stable."**
Everything else must come from the doctor.

- ⌚ Doctors lose clinical time on the phone.
- ⚠ Families left in the dark file **complaints, even lawsuits.**

Nobody is doing anything wrong. There's just no channel or time.

The solution

MindPeace turns the doctor's clinical daily note into a plain-language, doctor-approved, update, plus a voice agent that answers families 24/7 using only approved information.



Read it or ask it



Read

Ana checks the timeline between meetings.



Ask

António, 81, Maria's husband, taps one button and just talks. No reading, no typing.

Live demo

One morning, four screens

1

Doctor

A clinical note,
every line sorted

Shareable

Withheld

Internal

Then: approve.

2

Family

A push notification,
and a new timeline
node appears live.

3

Voice call

"When can I visit?"

Gets an answer.

"Can it get more
serious?"

A kind decline. The
question is logged, a
callback is booked.

4

Doctor again

The question is in
the queue.

*One list instead of
a morning of
phone calls.*

How it knows what not to say

We didn't invent the rules. We encoded the rules hospitals already follow.

Safe to share

Shared automatically. Stable, awake, vitals, ate well.

Needs approval

Doctor approves first. Diagnoses, results, the plan, the discharge date.

Care team only

Never shared. Instructions for the nursing team.

🔒 Nothing reaches a family without the doctor's click.



Dr. Inês Silva

Impact

Better for everyone around the bed



Families

An answer in 5 seconds

Any hour, in their language, by reading or just by asking.



Doctors

One question list

Instead of a morning of calls, and fewer complaints (and lawsuits).



Nurses

No more “I'm not allowed to tell you”

Business and vision

The hospital pays. Families use it for free.

First customers

Private hospitals

CUF, Luz, Lusíadas



Then

The public SNS

Portugal's national health service



Next

After discharge

Daily check-in calls that escalate warning signs

🔗 FHIR integration, with patient consent



One click for the doctor.
Peace of mind for the family.

mindpeace-health.vercel.app



Scan to try it

The questions you're probably asking



Where does the data come from?

The hospital's **electronic patient record**.

Pulled **once a day**, after the doctor updates the note.

Read-only through FHIR. We never write to the record.



Who gives consent?

The patient decides. At admission they get a **QR code or link** to share, or give us contacts and we send the invites.

The patient can **revoke access** at any time.

Patient can't consent (unconscious, dementia)? Their **legal representative** decides, the same rule the hospital already uses.



What about patient privacy?

Patient data **never reaches the AI with a name on it**.

A: local model on the hospital's own servers. Simple task, and small models improve every month.

B: pseudonymisation. Identifying details are removed before the AI sees the note and put back afterwards.

Under the hood

Claude filters the note and runs the agent with real tool calls

`get_approved_update`

`log_question_for_doctor`

`book_callback_slot`

ElevenLabs real-time voice (PT / EN / DE) · **Supabase** live sync · **Next.js** on Vercel

Cached fallbacks, so the demo can't crash

✓ **Real**

AI filter, approval, family app, voice agent

Simulated

Hospital records, login, calendar